

SURGICAL PATHOLOGY

Case Number :

Criteria	Yes	No
Diagnosis Discrepancy		☒
Primary Tumor Site Discrepancy		☒
HPAA Discrepancy		☒
Prior Malignancy History		☒
Dual/Synchronous Primary Noted		☒
Case is (circle):	QUALIFIED	DISQUALIFIED
Reviewer Initials	Date Reviewed: 12/19/11	

Diagnosis:

A: Lymph node, superior thyroid pole, removal
 - Consistent with metastatic papillary carcinoma of thyroid in 1 of 1 lymph node
 (1/1) with complete effacement of lymph node architecture; maximum size of metastasis 0.5 cm; negative for extracapsular extension.

B: Lymph node, superior thyroid pole, removal
 - Metastatic papillary carcinoma of thyroid in 1 of 1 lymph node (1/1); maximum size of metastasis 0.6 cm; focal extracapsular extension identified.

C: Thyroid gland, total thyroidectomy

Tumor histologic type: papillary carcinoma of thyroid, mixed follicular and papillary variants

Tumor size: 4.5 cm

Tumor growth pattern: infiltrative

Tumor focality: left lobe, unifocal

Extent of invasion:

Capsular Invasion: not applicable

Lymphovascular: not identified

Blood vascular: not identified

Extra thyroidal extension: present, focal extension into perithyroidal soft tissues (C4)

Surgical margins: carcinoma focally abuts the black inked posterior margin (C5), remaining margins negative for definite tumor

Status of thyroid gland away from tumor: unremarkable

Lymph nodes: separately submitted, total lymph node count for all specimens

ICB-0-3
 CQCF: carcinoma, papillary; thyroid, NOS 8260/3
 Path: Carcinoma, papillary + follicular 8340/3
 Site: thyroid, NOS C73.9
 12/19/11

UUID: C486F0BE-B25C-4108-81C5-AAC6B64346A0
 TCGA-DE-A3KN-01A-PR Redacted



is 5 of 9 lymph nodes involved by metastatic papillary carcinoma (5/9)

Size of largest metastasis (greatest dimension): 0.6

Extracapsular (perinodal) extension: focally present

AJCC PATHOLOGIC TNM STAGE: pT3 pN1b

NOTE: This pathologic stage assessment is based on information available at the time of this report, and is subject to change pending clinical review and additional information.

D: Central lymph nodes and thymus, thymectomy and lymph node removal

- Metastatic papillary carcinoma of thyroid in 3 of 7 lymph nodes (3/7); maximum size of metastasis 0.6 cm; negative for extracapsular extension
- Benign thymic tissue

Comment:

The frozen section diagnosis is confirmed.

Intraoperative Consult Diagnosis:

An intraoperative consultation was requested by Dr. from
at on

FSA1: Lymph node, superior thyroid pole, biopsy

- Follicular neoplasm with features suspicious for follicular variant of papillary carcinoma
- No lymphoid tissue present, but cannot exclude metastatic tumor
- Dr. concurs

Drs. at

Frozen Section Pathologist:, MD

Clinical History:

-year-old female with left thyroid nodule. FNA shows malignancy favoring papillary carcinoma.

Gross Description:

Specimen A is received in one appropriately labeled containers, additionally labeled "superior thyroid pole lymph node" and holds a 0.4 x 0.2 x 0.1 cm pink/tan soft tissue fragment that is entirely frozen as FSA1,

Specimen B is received in one appropriately labeled container, additionally labeled superior thyroid pole lymph node 2 and holds a white/tan lymph node candidate that measures 0.5 x 0.5 x 0.3 cm. The lymph node candidate is sectioned and entirely submitted in block B1,

Specimen C:

Specimen fixation: Formalin

Type of specimen: Total thyroidectomy

Size and weight of specimen: Overall 7.2 x 5.4 x 2.8 cm; right lobe (4.4 x 2.2 x 2.4 cm); left (5.2 x 4.2 x 2.8 cm); weight: 29 grams.

Orientation: Anterior/blue, posterior/black, isthmus/yellow. The specimen is oriented noting that the tumor is located in the left lobe.

Tumor location: Left lobe

Focality: Unifocal

Tumor description: A very firm solitary unencapsulated mass (4.5 x 2.5 x 2.0 cm) is noted expanding the mid portion of the left thyroid lobe. The cut surface is yellow/tan, solid and lobulated with scattered calcifications. No areas of hemorrhage or necrosis are grossly observed. The mass appears well-circumscribed with compression of the adjacent residual thyroid parenchyma.

Tumor size: 4.5 x 2.5 x 2.0 cm

Confinement/non-confinement: Confined

Distance of tumor to surgical margins: The mass grossly abuts the blue inked and black inked anterior and posterior margins, respectively. No extension into the isthmus is grossly identified.

Appearance of thyroid gland away from tumor: The remainder of thyroid parenchyma in the left lobe is red/tan and compressed. The isthmus and right thyroid lobe are grossly unremarkable.

Parathyroids: Not identified

Tissue submitted for special investigations: Tumor was collected for tissue procurement.

Lymph nodes: N/A

Digital photograph taken: N/A

Block Summary:

C1-C5 - Representative mass with margins and adjacent thyroid

C6 - Left thyroid lobe at edge of tumor, in continuity with grossly identified tumor

C7 - Isthmus

C8-C9 - Representative sections of right thyroid lobe

Specimen D is received in one appropriately labeled container, additionally labeled "central lymph nodes and thymus" and holds a firm, nodular pink/tan tissue fragment that measures 4.0 x 2.2 x 0.3 cm. The tissue fragment has a matted appearance and with gentle palpation separates into three tissue fragments. The largest fragment is composed of grey-tan lobulated tissue and presumed to represent thymic tissue. The remaining fragments are presumed to represent the "central lymph nodes" and submitted per block

summary.

Block Summary:

D1-D2 - ? thymus

D3 - One candidate lymph node, sectioned

D4 - One candidate lymph node, sectioned